



**Citation: Oliveira v. Aviva General Insurance, 2025 ONLAT 22-008987/AABS**

**Licence Appeal Tribunal File Number: 22-008987/AABS**

In the matter of an application pursuant to subsection 280(2) of the *Insurance Act*, RSO 1990, c I.8, in relation to statutory accident benefits.

Between:

**Rebecca Oliveira**

**Applicant**

and

**Aviva General Insurance**

**Respondent**

**DECISION**

**ADJUDICATOR: Tanjoyt Deol**

**APPEARANCES:**

For the Applicant: Sherilyn Pickering, Counsel

For the Respondent: Camilla Oblak, Counsel

**HEARD: By way of written submissions**

## OVERVIEW

- [1] Rebecca Oliveira, (the “applicant”), was involved in an automobile accident on December 8, 2020, and sought benefits pursuant to the *Statutory Accident Benefits Schedule - Effective September 1, 2010 (including amendments effective June 1, 2016)* (the “Schedule”). The applicant was denied benefits by Aviva General Insurance (the “respondent”), and applied to the Licence Appeal Tribunal - Automobile Accident Benefits Service (the “Tribunal”) for resolution of the dispute.

## ISSUES

- [2] The issues in dispute are:
1. Are the applicant’s injuries predominantly minor as defined in s. 3 of the *Schedule* and therefore subject to treatment within the \$3,500.00 Minor Injury Guideline (“MIG”) limit?
  2. Is the applicant entitled to \$2,045.38 for an occupational therapy (“OT”) assessment, proposed by Rehab First Inc., in a treatment plan/OCF-18 (“OCF-18”) dated April 16, 2021?
  3. Is the applicant entitled to \$4,150.00 for medical services (MRI and SPECT Scan) proposed by East York Physiotherapy and Orthopaedic Rehabilitation Clinic in a OCF-18 dated May 7, 2021?
  4. Is the applicant entitled to \$16,272.00 for catastrophic impairment assessments proposed by East York Physiotherapy and Orthopaedic Rehabilitation Clinic in a OCF-18 dated September 19, 2021?
  5. Is the applicant entitled to interest on any overdue payment of benefits?
  6. Is the respondent liable to pay an award under s. 10 of Reg. 664 because it unreasonably withheld or delayed payments to the applicant?
- [3] I note that the Case Conference Report and Order (“CCRO”), released on May 12, 2023, noted that for issue four, the amount in dispute was \$36,560.00. However, the applicant in her submissions has conceded that she is seeking \$16,272.00 which encompasses \$200.00 for the OCF-18, \$2,000.00 per each assessment, and \$200.00 for the OCF-19, and HST. As such, the amount in dispute for issue four is \$16,272.00 which has been reflected above.

## RESULT

[4] I find that:

- a. The applicant is removed from the MIG;
- b. The applicant is entitled to the OCF-18s for an OT assessment and for the catastrophic impairment assessments plus interest;
- c. The applicant is not entitled to the OCF-18 for medical services (MRI and SPECT Scan); and
- d. The applicant is entitled to an award in the amount of 25 percent, plus interest pursuant to s. 10 of Reg. 664 for the OCF-18s for an OT assessment and for the catastrophic impairment assessments.

## PROCEDURAL ISSUES

### ***The applicant's motion to exclude s. 44 reports and Rule 10.4***

- [5] The applicant's motion to exclude certain s. 44 reports from the hearing record is granted in part.
- [6] On January 30, 2024, the applicant filed a Notice of Motion requesting that the following s. 44 reports be excluded from the hearing record:
- A. Neurology Assessment Report and Neurology Addendum Report of Dr. Paul J. Ranalli, neurologist; dated August 27, 2021 and August 3, 2023;
  - B. Orthopaedic Surgery Assessment Addendum of Dr. Erin Lynn Boynton, orthopaedic surgeon, dated December 7, 2023; and
  - C. Psychology Assessment Report by Dr. Nicole Azizli, psychologist, dated December 7, 2023.
- [7] The applicant makes this request because, according to the applicant, these reports (with the exception of Dr. Ranalli's reports) were served late and are non-compliant with Rule 10.2(b) of the *Licence Appeal Tribunal Rules, 2023* ("The Rules"). The applicant also argues that if the Tribunal admits these reports, then they should be given reduced and limited weight and that Drs. Ranalli, Boynton and Azizli be considered as participant experts.
- [8] The applicant also requested that the deadline under Rule 10.4 of the *Rules* be waived so she can challenge the reports set out above on the basis that no

Acknowledgement of Expert Duty (“AED”) was filed. Alternatively, if the reports are admitted into the record, she requests that the Tribunal give limited weight to these reports due to the non-compliance with the Case Conference Report and Order (“CCRO”), released on May 12, 2023, and the failure to file an AED.

- [9] The respondent argues that it served various productions in compliance with the CCRO including the Curriculum Vitae (“CV”) of all s. 44 assessors. It further argues that the applicant late served the s. 25 report of Dr. Grigory Karmy, chronic pain physician, dated June 24, 2023, and the psychovocational report of Dr. Phillip Miller, psychologist, and Mr. Allan Walton, psychotherapist, dated June 27, 2023, and that she did not serve the CV of Dr. Karmy or the AED/CV of Dr. Miller/Mr. Walton. As a result, the respondent argues that if its reports are excluded, then the applicant’s expert evidence should also be found inadmissible.

#### **A) Rule 10.4**

- [10] I find that the applicant has established that an extension of one day for the deadline under Rule 10.4 of the *Common Rules of Practice and Procedure* (October 2017) (“the *Common Rules*”) is warranted.
- [11] The applicant argues that the deadline under Rule 10.4 of the *Rules* be waived as 21 days notice was not available between the filing of the respondent’s submissions and the hearing date of February 9, 2024.
- [12] The respondent argues that it would be unjust for the Tribunal to selectively enforce Rule 10.2 but not Rule 10.4. It argues that the assessors were not a surprise to the applicant because it expressed its intention to rely on their expert reports in its Case Conference Report and Summary.
- [13] I acknowledge that both parties made extensive submissions on Rule 10.2 and 10.4 of the *Rules* being applicable. However, in this case, Rule 10 of the *Common Rules* apply because under Rule 10.5 of the *Rules*, it states Rule 10 only applies to appeals or where the first notice of case conference is issued on or after August 21, 2023. Here, the case conference took place on May 10, 2023, which means that neither the appeal nor first notice of case conference was issued after August 21, 2023.
- [14] The deadline under Rule 10.4 of the *Common Rules* is 10 days for any party to challenge the expert’s witnesses, qualifications, reports, or statements. As the applicant filed her motion material on January 31, 2024, and the hearing was

scheduled for February 9, 2024, she was non-compliant with Rule 10.4 of the *Common Rules* because she gave notice nine days before the hearing.

- [15] In accordance with Rule 3.1(a) of the *Rules*, I must ensure that the *Rules* are liberally interpreted to ensure procedural fairness to both parties, and the efficient, proportional, and timely resolution of the merits of the proceedings before the Tribunal.
- [16] When weighting procedural fairness and any potential prejudice brought, I find that the applicant would be severely prejudiced if an extension was not granted because she was a day late in the deadline and Dr. Boynton and Dr. Azizli's updated reports were late served. Moreover, the respondent has not referred me to evidence that it advised the applicant that it intended to rely upon these updated reports prior to submitting its responding submissions. I acknowledge the respondent's position that it advised the applicant of its intention to call these assessors in the Case Conference Report and Summary, however that was in relation to their previous assessment of December 2, 2021, and not the updated assessment that took place on December 7, 2023. With respect to Dr. Ranalli's report, the applicant's notice is only one day late, and the respondent did not provide specific submissions as to the prejudice it would suffer if an extension of one day was granted. The applicant also brought her motion within four days of receiving the respondent's submissions, therefore I find that she took steps to provide notice in a timely fashion.

**B) Dr. Ranalli's reports**

- [17] The applicant's request to exclude Dr. Ranalli's reports, dated August 27, 2021 and August 3, 2023, from the hearing record is denied.
- [18] I acknowledge that the respondent did not file an AED for Dr. Ranalli until February 7, 2024, which is non-compliant with Rule 10.3 of the *Common Rules*. However, the *Common Rules* are to be liberally interpreted and applied and may be varied to facilitate a fair process. Here, excluding Dr. Ranalli's reports would result in significant prejudice to the respondent because it would not be able to rely on the reports which contain evidence that is relevant to the issues in dispute, being the MIG and the OCF-18s. I have not been provided with evidence to suggest that Dr. Ranalli is not qualified to provide expert evidence in this matter and where potential exists for significant prejudice to one of the parties, procedural fairness mitigates in favour of admitting the evidence.
- [19] When weighing procedural fairness and any potential prejudice brought, I find the scales tip in favour of the respondent. The respondent would be unfairly

prejudiced if the reports of Dr. Ranalli were excluded from the record as it relies upon these reports for the issues in dispute. However the lack of filing an AED within the deadline mandated under Rule 10.3 of the *Common Rules* will go to the weight I assign to these expert reports, which is also an alternative relief sought by the applicant.

- [20] Dr. Ranalli will still be considered an expert witness and not as a participant expert. The applicant argues that in *Handy v. Aviva Insurance Company of Canada*, 2022 CanLII 78793 (ON LAT) (“*Handy*”) where a breach of Rule 10.2 had occurred, the adjudicator declined to accept the assessor as an expert and instead accepted them as a participant expert. In my opinion, the factual matrix before me is distinguishable from *Handy* because the respondent advised the applicant in its case conference summary that it intended to rely upon the evidence of Dr. Ranalli for the hearing and she was provided with the reports within the deadline mandated by the CCRO. Meanwhile, the applicant in *Handy* did not have notice that the respondent intended to call the expert witness, nor did the applicant have the records from the witness that would have acted as summary of their anticipated evidence.
- [21] The applicant’s argument that Dr. Ranalli did not sign either of his reports is without merit, because both reports contain his signatures. It is unclear from the applicant’s submissions what relief she is seeking in regard to this alleged non-compliance; however, it is immaterial because Dr. Ranalli signed both reports.
- [22] For all these reasons, I decline to exclude Dr. Ranalli’s reports dated August 27, 2021 and August 3, 2023 from the hearing record and Dr. Ranalli will still be considered an expert witness, and not a participant expert.

**C) Dr. Boynton’s report**

- [23] The applicant’s request to exclude Dr. Boynton’s report dated December 7, 2023, from the hearing record is denied.
- [24] The applicant requested Dr. Boynton’s report be excluded due to non-compliance with the deadline mandated by the CCRO and/or for the failure to provide an AED by the deadline mandated under Rule 10.3.
- [25] The CCRO was clear that the parties exchange all documents that have not been previously exchanged no later than 60 calendar days following the case conference, being July 10, 2023 (both parties disagree over whether this deadline falls on July 9 or 10, which will be discussed below). The CCRO also ordered that the parties disclose any additional items responsive to items already

produced that they intended to rely upon at the hearing no later than August 10, 2023.

- [26] The respondent was non-compliant with the CCRO because Dr. Boynton's report was not produced until December 8, 2023, and the deadline under the CCRO was August 10, 2023 because this report was responsive to items already produced. However, I find that Dr. Boynton's report is relevant to the issues in dispute, despite the respondent being in breach of the CCRO. Pursuant to s. 15(1)(b) of the *Statutory Powers Procedure Act*, RSO 1990, c S. 22 ("SPPA"), documents relevant to the issues in dispute are admissible as evidence.
- [27] I am not persuaded by the applicant's position that she will suffer prejudice if Dr. Boynton's report is admitted into the record. The applicant argues that there is presumed prejudice and that she did not have time to prepare rebutting reports because the report was late served by the respondent. Significantly, the report prepared by Dr. Boynton is four pages long, where she reviewed the updated medical documentation and concluded that this information did not change her conclusion in her original report.
- [28] I disagree that there is prejudice because the applicant was in possession of the original report prepared by Dr. Boynton, dated December 2, 2021, and therefore had the opportunity to prepare a rebuttal report through Dr. Karmy. However, Dr. Karmy did not review Dr. Boynton's report in his report dated June 14, 2023. Meanwhile, the report of Dr. Boynton, dated December 7, 2023, she maintained her original opinion, and therefore this does not constitute as a new opinion.
- [29] I further acknowledge the parties' reliance on *Certas Direct Insurance Company v. Gonsalves*, 2011 ONSC 3986 ("Gonsalves") which they both argue supports their position. In *Gonsalves*, the Court considered new evidence, where one month prior to the commencement of the arbitration the claimant delivered two new orthopedic opinions. The Court noted that this left the insurer with no practical ability to respond to the opinions and found that such a denial of the right to make a full response was a violation of procedural fairness.
- [30] While I am bound by *Gonsalves*, the factual scenario before me is distinguishable because here the respondent is not producing a new opinion by Dr. Boynton. Rather, as indicated above, Dr. Boynton, in her report dated December 7, 2023, is maintaining her original opinion despite receiving updated medical information. Therefore, I am not persuaded by the applicant's position that there is prejudice by admitting Dr. Boynton's report dated December 7, 2023 because there is no trial by ambush where an expert maintained their original opinion.

- [31] The applicant also argues that she has not had the opportunity to cross-examine Dr. Boynton. According to the CCRO, the parties agreed to a written hearing and that the only cross-examination would occur was for the applicant's affidavit evidence. Thus, it is unclear to me how the applicant was denied an opportunity for a cross examination when as per the CCRO, she agreed to proceed through a written hearing format where only a cross examination of the applicant's affidavit evidence was agreed to.
- [32] The applicant argues that Dr. Boynton's report dated December 7, 2023 is unsigned. I disagree because the report contains his signature. It is again unclear from the applicant's submissions what relief she is seeking in regard to this alleged non-compliance; however, it is immaterial because Dr. Boynton signed the report.
- [33] I will not exclude Dr. Boynton's report, dated December 7, 2023, on the basis that no AED was filed until February 7, 2024. I find that the respondent was non-compliant with Rule 10.3 of the *Common Rules* because it did not file the AED within 20 days of the hearing. However, an exclusion of the report would result in significant prejudice to the respondent because it is relevant to the issues in dispute. Moreover, if any prejudice exists to the applicant it is mitigated because she was able to address the report in her reply submissions.
- [34] In a similar vein, I will not grant the applicant's relief to consider Dr. Boynton as a participant expert because she was aware that the respondent intended to rely upon Dr. Boynton's original report as evidence because it is indicated in the case conference summary of the respondent. As already noted above, Dr. Boynton in her updated report maintained her original opinion. However, I will grant the applicant's relief to afford little weight to Dr. Boynton's report due to non-compliance with the CCRO and Rule 10.2 of the *Common Rules*.
- [35] To conclude, I will not exclude Dr. Boynton's report, dated December 7, 2023 from the record, and Dr. Boynton will still be considered a expert witness and not a participant expert.

**D) Dr. Azizli's report**

- [36] The applicant's request to exclude Dr. Azizli's report dated December 7, 2023 from the hearing record is granted. To be clear, Dr. Azizli had another previous report, dated December 2, 2021, however, the applicant's Notice of Motion and motion materials indicated that it was only seeking to exclude the report dated December 7, 2023. Therefore my ruling only pertains to the report, dated December 7, 2023.



- [37] The deadline to file this report was August 10, 2023, and this report was filed four months past the deadline. Dr. Azizli's report contains new information and opinions that are attributed to the applicant being a new mother and her activities of daily living. The applicant has not had an opportunity to have a rebuttal report completed in response to these new opinions.
- [38] I further acknowledge the respondent's position that the s. 44 assessment was conducted upon receipt of the applicant's updated medical documentation which was produced after the deadline in the CCRO. The respondent did not specify on which updated medical documentation was provided and when. Further, the respondent argued that the applicant produced Drs. Karmy, and Miller and Mr. Walton's report one day past the deadline but this is incorrect. The 60 day deadline as mandated under the CCRO fell on a holiday, being July 9, 2023, which was a Sunday. Therefore, the applicant is correct that she had until July 10, 2023, to serve these reports, which she did.
- [39] In any event, Dr. Azizli's report referenced not only these s. 25 reports but a number of clinical notes and records ("CNRs"), which again I do not know when these were served. Furthermore, the respondent took no action in bringing a motion prior to the submissions being due to admit Dr. Azizli's report into the record.
- [40] Although I acknowledge the prejudice to the respondent if Dr. Azizli's report was excluded, the prejudice to the applicant if the report of Dr. Azizli is admitted cannot be cured within this hearing because the time to disclose documents and make submissions has expired. In accordance with *Gonsalves*, I find that to admit the evidence of Dr. Azizli would rise to a breach of procedural fairness because it is a new medical opinion that the applicant has not had an opportunity to respond to.
- [41] Finally, against the respondent's argument, the applicant was compliant with both the CCRO and Rule 10.2, because she served the CVs, AEDs, and the s. 25 reports on July 10, 2023. As noted above, the 60 day deadline as per the CCRO fell on July 9, 2023, which is a Sunday, therefore in accordance with Rule 5.2 of the *Rules*, the deadline for these reports was on the next day, which is not a holiday, which is July 10, 2023. Moreover, the applicant produced a copy of the CVs and AED for all its assessors on July 10, 2023, and therefore was compliant with Rules 10.2 and 10.3 of the *Common Rules*.
- [42] For all these reasons, the applicant's request to exclude Dr. Azizli's report dated December 7, 2023 is granted.

## ANALYSIS

### ***The applicant is removed from the MIG because she sustained hearing loss in her left ear***

- [43] I find that the applicant has established that she was diagnosed with hearing loss in the left ear as a result of this accident by Dr. Randy Leung, otolaryngologist, on December 14, 2020, which removes her from the MIG.
- [44] Section 18(1) of the *Schedule* provides that medical and rehabilitation benefits are limited to \$3,500.00 if the insured person sustains an impairment that is predominantly a minor injury. Section 3(1) defines a “minor injury” as “one or more of a sprain, strain, whiplash associated disorder, contusion, abrasion, laceration or subluxation and includes any clinically associated sequelae to such an injury.”
- [45] An insured person may be removed from the MIG if they can establish that their accident-related injuries fall outside of the MIG or, under s. 18(2), that they have a documented pre-existing injury or condition combined with compelling medical evidence stating that the condition precludes recovery from any accident-related minor injury if they are kept within the confines of the MIG. The Tribunal has also determined that chronic pain with functional impairment or a psychological condition may warrant removal from the MIG. In all cases, the burden of proof lies with the applicant.
- [46] The applicant submits that she should be removed from the MIG for five reasons including the fact that she has hearing loss in her left ear and tinnitus. The applicant argues that hearing loss and tinnitus do not fall within the MIG and she relies upon the CNRs of Dr. Leung, and the s. 44 neurology assessment report of Dr. Ranalli, dated August 27, 2021 to support these diagnoses.
- [47] To counter, the respondent argues that the applicant sustained soft tissue type injuries that fall within the MIG. Further the respondent provided no submissions on how the applicant could remain in the MIG when she sustained hearing loss in her left ear. Instead, it summarized the s. 44 report of Dr. Ranalli dated August 27, 2021, which noted that the applicant had sustained traumatic transient sensorineural hearing impairment on the left side, and that the applicant was recovering from loss of hearing.
- [48] I agree with the applicant that hearing loss does not fall within the MIG because it is not a sprain, strain, whiplash associated disorder, contusion, abrasion, laceration, or subluxation. Hearing loss is also not a clinically associated

sequelae to such an injury since it is a separate impairment and not associated to the above.

- [49] I also place weight on the December 14, 2020 CNR in which Dr. Leung diagnosed the applicant with left hearing loss after concussive noise exposure. Enclosed in Dr. Leung's CNR was a copy of a hearing test report, dated December 14, 2020, conducted by Ms. Jennifer Anderson, audiologist and reviewed by Dr. Leung. In this hearing test report, it was opined by Dr. Leung and Ms. Anderson that the applicant sustained mild sensorineural hearing loss in the left ear.
- [50] Further, Dr. Ranalli concluded that the applicant sustained traumatic transient sensorineural hearing impairment on the left side, but he did not opine that this was a soft tissue injury. Instead, Dr. Ranalli concluded that from a neurological perspective that the applicant's injuries can be described as a minor injury, however he provided no rationale on how hearing loss in the left ear would fall in the MIG definition.
- [51] The fact that Dr. Ranalli opined that the applicant was recovering from her hearing loss is immaterial to the issue of whether hearing loss is within the definition of the MIG or not. There is no requirement under s. 3(1) for the applicant to be continuously impaired from an impairment, instead the issue is whether the applicant's impairment fell within the definition of minor injury or not. Here, as noted above, hearing loss is not captured within the definition of a minor injury, and it is irrelevant whether the applicant was recovering or not from such an impairment.
- [52] I am alive to the respondent's position that the applicant sustained soft tissue injuries which are captured within the MIG. I disagree as hearing loss is not a soft tissue injury, nor has the respondent referred me to evidence to support such a proposition.
- [53] The fact that the applicant's hearing loss might have improved, does not mean that the applicant's injury is in the MIG. To give a parallel example, if an applicant broke a bone as a result of the accident, and later it heals, does not mean the applicant should be considered in the MIG. If at any point the applicant is diagnosed, by a doctor capable of making that specific diagnosis, of an injury that is not within the MIG, then it warrants removal.
- [54] Finally, I acknowledge the applicant's argument that tinnitus is also not covered within the MIG, however as I have determined that the applicant's hearing loss

diagnosis is sufficient to remove her from the MIG, I do not need to consider this argument.

- [55] In conclusion, the applicant has established on a balance of probabilities that she sustained hearing loss in her left ear, which is not covered within the definition of “minor injury” under the *Schedule*. As a result, she is removed from the MIG.
- [56] As I have decided that the applicant is removed from the MIG on the basis of hearing loss, I do not need to consider the other grounds the applicant advanced to remove her from the MIG.

***The applicant is entitled to the OCF-18 for an OT assessment***

- [57] I find that the applicant has proven on a balance of probabilities that the proposed OT assessment is reasonable and necessary.
- [58] To receive payment for a treatment and assessment plan pursuant to sections 15 and 16 of the *Schedule*, the applicant bears the burden of demonstrating on a balance of probabilities that the benefit is reasonable and necessary as a result of the accident. To do so, the applicant should identify the goals of treatment, how the goals would be met to a reasonable degree and that the overall costs of achieving them are reasonable.
- [59] The goals of the OCF-18 are to assess the applicant’s current level of functioning, safety, and independence with her pre-collision activities of normal living within her home and community environment, for her to return to her activities of normal living, and to assess and facilitate a safe functional participation in her activities of normal living. In essence, the purpose of the OCF-18 is to determine what accident-related restrictions the applicant has to her daily activities, and to provide assistance in order for the applicant to return to these activities.
- [60] In support of the OCF-18, the applicant relies upon a screening assessment completed by Ms. Lauren Schwalm, occupational therapist, which was attached to the OCF-18. Based on her screening assessment, Ms. Schwalm concluded that the applicant’s impairments were affecting her ability to carry out her tasks of employment and activities of normal living.
- [61] The respondent argues that the OT assessment is not reasonable and necessary because the applicant has returned to work, and returned to all activities of daily living. It further argues that the screening assessment conducted by Ms. Schwalm should be given little weight because it is based on the applicant’s self-

reporting and that no medical documentation was reviewed. Instead, it argues that the report of Dr. Boynton, dated December 2, 2021 and December 7, 2023, and Dr. Azizli's report, dated December 2, 2021, should be preferred by the Tribunal.

- [62] Ms. Schwalm's opinion, however, is supported by the bulk of the evidence before me, and I have no reason to doubt the applicant's self-reporting. Indeed, the applicant has consistently reported in her affidavit evidence, cross examination, and the s. 25 reports of Drs. Karmy, Miller, and Mr. Walton that she has functional limitations with her recreational activities, such as snowboarding, hockey, and skating, and heavier housekeeping tasks such as lifting heavy boxes, moving furniture to vacuum and clean, gardening and shoveling snow. Therefore, it is reasonable to investigate how these limitations can be treated through OT services.
- [63] Moreover, the applicant reported to Drs. Miller, and Mr. Walton, that she took approximately five to six months off work at her pre-accident employment as a early childhood assistant at a daycare, and when she was ready to return to her role, the position had been filled. As a result, the applicant decided to pursue esthetics training and worked in a salon, until she opened her own store in September of 2022. However, the applicant also reported that because of her neck pain she was at best able to work seven-hour shifts, and that she believed that she would be able to have more clients if she wasn't involved in the accident.
- [64] The applicant also reported to Dr. Karmy that she had pain from working as an esthetician because she was required to be on her feet for extended periods of time, bend forward, twist repetitively, sustain a stooped position, and perform repetitive upper limb movements and reaching. As a result, she reported to Dr. Karmy that she has lost some of her income because of reduced job performance.
- [65] In both her cross examination and affidavit evidence, the applicant testified that following the accident, and before the pregnancy, she had difficulties with completing heavy housekeeping tasks, and has not gone snowboarding, ice skating, or played hockey since the accident. I have no reason to doubt the applicant's self-reporting where it has been consistent, and Dr. Miller, Mr. Walton and Dr. Azizli, in her report, dated December 2, 2021, have all opined that the applicant did not exhibit behaviour consistent with malingering and that she was candid.
- [66] In short, where the applicant has consistently reported limitations with her employment, housekeeping tasks, and recreational activities, such as

snowboarding, hockey and skating, I find that it is reasonable and necessary for an OT assessment to be conducted to investigate how these limitations can be treated through OT services.

- [67] I also find that the reports of Drs. Boynton and Azizli (dated December 2, 2021) have limited evidentiary value for the following reasons.
- [68] First, Dr. Boynton provided a limited rationale on how she concluded that the applicant was independent in her activities of daily living or whether she asked the applicant about her difficulties with completing heavier housekeeping tasks, and not returning to hockey, skating, and snowboarding. In her report, Dr. Boynton noted that the applicant is independent but there is no indication of how the conclusion arose. While Dr. Boynton noted that the applicant was able to do her cooking and cleaning, she also reported neck and shoulder pain with her work in an esthetic salon, left shoulder pain with repetitive movement overhead or lifting overhead, and that her shoulder and neck pain were aggravated by picking up heavy items.
- [69] Second, Dr. Boynton opined that the applicant had excellent mobility of the cervical spine and excellent strength of her muscles and, therefore, an OT assessment was not reasonable and necessary. However, the applicant has been diagnosed with a head injury by Dr. Maurice Levitan, neurologist, Dr. Karmy, and Dr. Ranalli. Dr. Karmy also opined that the applicant's headaches impacted her daily activities, and work. Therefore, the applicant is not limited in her functional activities solely due to her physical impairments but also because of her headaches.
- [70] Third, Dr. Azizli (in the report, dated December 2, 2021) concluded that the applicant had returned to all activities of daily living, including returning to gainful employment, yet the applicant reported that she has not returned to snowboarding since the accident, she was terminated as an Early Childhood Educator Assistant following this accident, and that she had pain in her current occupation. The applicant reported that she was currently working in an esthetic salon where she mainly does lashes and brows. The applicant also reported neck and shoulder pain after a long day of looking down at work.
- [71] Fourth, Dr. Boynton in her updated report concluded that any pain symptoms that the applicant is currently experiencing is related to poor posture and myofascial pain unrelated to the accident but again provides limited reasoning on how such a conclusion was reached. In his previous report, dated December 2, 2021, Dr. Boynton noted that the applicant had muscular discomfort in the trapezius that was associated with head forward posture and decreased thoracic spine mobility,

and he provided her with exercises. Meanwhile, in her updated report, dated December 7, 2023, Dr. Boynton does not address where the applicant has poor posture, i.e., is it the entire back or head forward posture. Dr. Boynton also does not address how she arrived at the conclusion that the applicant's pain symptoms were related to poor posture, when she did not speak to her since the previous assessment.

- [72] Fifth, as noted above, no AEDs for Dr. Boynton and Dr. Ranalli were filed in accordance with the timeline under 10.3 of the *Common Rules* and the updated report of Dr. Boynton, dated December 7, 2023, was late served. As a result, I also place limited weight to Dr. Boynton and Dr. Ranalli's reports on this basis.
- [73] For all these reasons, the applicant has established that the proposed OT assessment is reasonable and necessary.

***The applicant is not entitled to the OCF-18 for an MRI and SPECT Scan***

- [74] I find that the applicant has not proven on a balance of probabilities that the OCF-18 proposing a MRI and SPECT scan is reasonable and necessary.
- [75] This OCF-18 is in relation to: documentation, support activity, completion of an MRI and SPECT scan, and results review and report writing by Dr. Yin-Hui Siow, radiologist. The goals of which are to make recommendations for future care.
- [76] The applicant argues that she was diagnosed with a traumatic brain injury by Dr. Leung, Dr. Levitan, and Dr. Ranalli and that the MRI and SPECT scan were required to rule out brain pathology, examine disruption of the anatomy due to injury and to examine the disruption of brain function. She further argues that the use of SPECT as a secondary tool to confirm the diagnosis, assess the functionality of the brain, and assist with future care is consistent with *Wabie v. Wilson*, 2022 ONSC 4296 ("*Wabie*").
- [77] The respondent argues that both the SPECT scan and MRI are not reasonable and necessary because:
- a. Dr. Levitan remarked that SPECT scan results were suspicious; and
  - b. Dr. Ranalli opined that SPECT scan technology is unreliable in the setting of a mild traumatic brain injury.
- [78] I find that the SPECT scan was not used as a secondary tool as proposed by the applicant because there was no diagnosis of a traumatic brain injury until August 16, 2021, which is after the testing was completed. A SPECT scan was

completed on June 10, 2021, yet there was no diagnosis of a traumatic brain injury until Dr. Ranalli's report, which was two months after the scan was completed.

- [79] Further, I disagree with the applicant that she was diagnosed with a traumatic brain injury by Dr. Leung. On December 14, 2020, Dr. Leung in his CNRs diagnosed the applicant with hearing loss after concussive noise exposure, but there is no diagnosis of a traumatic brain injury. Likewise, in Dr. Leung's CNRs dated February 21, 2021, Dr. Leung made no diagnosis of a traumatic brain injury and instead noted that the hearing was unchanged. While Dr. Levitan and Dr. Karmy have both diagnosed the applicant with a traumatic brain injury, this was after the SPECT scan was completed.
- [80] In *Wabie*, the Court determined that the SPECT scan should not be excluded because it is being used as a secondary tool to support the diagnosis and symptoms known prior to the scan. The Court also noted that the plaintiff in that case was not using the SPECT scan as either a primary diagnostic tool or as a means to distinguish depression and anxiety from a traumatic brain injury.
- [81] As noted above, the SPECT scan was conducted before a diagnosis of a traumatic brain injury was made and therefore it is being used as a primary rather than a secondary tool. Moreover, in the SPECT scan summary, Dr. Yin-Hui Siow, radiologist used the results of the SPECT scan to diagnose the applicant with anxiety disorders and depression in addition to the findings of a traumatic brain injury. The Court in *Wabie* at paragraph 144 noted that the Court has previously determined that the use of a SPECT scan to differentiate a traumatic brain injury from anxiety disorders and depression is novel and unreliable.
- [82] I find that *Wabie* is also distinguishable from the matter before me because it was a dispute over a Tort Claim and not accident benefits under the *Schedule*.
- [83] Even after I place reduced weight on Dr. Ranalli's report because no AED was filed in accordance with the timeline under 10.3 of the *Common Rules*, I still prefer the evidence of Dr. Ranalli over the evidence of Dr. Melanie Castre, physician. This is because Dr. Ranalli's opinion is also collaborated by the evidence of Dr. Levitan, the applicant's treating neurologist. I also place more weight on the evidence of Dr. Levitan and Dr. Ranalli because both are neurologists, and have more knowledge over the reliability of the SPECT scan than Dr. Castre, who is a physician.
- [84] Moreover, both Dr. Levitan and Dr. Ranalli have provided a persuasive rationale on why the use of a SPECT scan is not reliable. On April 18, 2022, Dr. Levitan



opined that he is suspicious of SPECT scans because he has seen this type of report in other individuals where they were young and perfectly healthy patients with normal MRIs. Likewise, Dr. Ranalli opined that the use of SPECT scan technology has shown to be unreliable. Meanwhile, Dr. Castre in the OCF-18 noted that further testing was required to rule out brain pathology but provides no compelling rationale on why a SPECT scan is reliable when it is being used as a primary tool and to distinguish anxiety disorders and depression from a traumatic brain injury. Instead, both Dr. Castre and the applicant refer to a number of studies on the SPECT scan, but what is lacking is the connection between those studies and the applicant's factual scenario.

- [85] The applicant has also provided no specific submissions on why the MRI of the brain is reasonable and necessary, especially where a previous CT scan was normal. The respondent also argues that the testing is available through OHIP and, therefore, not payable under s. 47(2) of the *Schedule*, which I agree with because MRIs are covered through OHIP.
- [86] While the applicant submitted that these services could not have been done through OHIP and relies upon the OCF-18 completed by Dr. Castre to support this position, Dr. Castre opined that the request could not be done through OHIP because both tests would not be completed on the same day and the results would not be synthesized into a comprehensive report. However, the applicant has provided no submissions on why both tests had to be done on the same day or why a comprehensive report was required, other than to state that the expertise of Dr. Siow would not have been available. Therefore, I find that the MRI of the brain was reasonably available through OHIP and also not payable under s. 47(2) of the *Schedule*.
- [87] For all these reasons, the applicant is not entitled to the OCF-18 proposing a MRI and SPECT scan.

***The applicant is entitled to the OCF-18 for a catastrophic impairment ("CAT") assessments***

- [88] I find that the applicant has proven on a balance of probabilities that the OCF-18 for the proposed CAT assessments is reasonable and necessary.
- [89] The OCF-18 seeks \$200.00 for the completion of the OCF-18 and \$200.00 for the completion of the OCF-19. The OCF-18 is also seeking \$2,000.00 each for the following assessments:
  - a. Catastrophic otorhinolaryngology report;

- b. Catastrophic In community situational occupational therapy assessment;
- c. Catastrophic In-Home OT assessment;
- d. Catastrophic neurological assessment;
- e. Catastrophic psychiatric assessment;
- f. Catastrophic orthopaedic assessment; and
- g. Catastrophic AMA Rating Report.

- [90] The applicant claims entitlement to the CAT assessments to explore whether she is catastrophically impaired and submits that there is a reasonable chance that she is catastrophically impaired as a result of the accident. The applicant further argues that the assessments are required to determine whether the 55% Whole Person Impairment (“WPI”) test or the mental/behavioural impairments in any of the four spheres of functioning meet the catastrophic impairment threshold.
- [91] The respondent argues that the applicant sustained soft tissue injuries from the subject accident and is classified under the MIG.
- [92] As noted above, s. 44 assessor, Dr. Ranalli concluded that the applicant sustained hearing loss and a traumatic brain injury following the accident. While Dr. Ranalli concluded in his August 27, 2021, report that the applicant’s frontal closed head injury, Grade 1 or mild Grade 2 had resolved, he provided no explanation on how he arrived at the conclusion of the applicant’s traumatic brain injury being resolved. Dr. Ranalli also noted that the applicant continued to have headaches, which is consistent with her complaints to several other assessors, and a visual aura, yet Dr. Ranalli concluded that the applicant’s traumatic brain injury had resolved. This conclusion is also inconsistent with Dr. Levitan prescribing a trial of Almotriptan for the applicant’s headaches in April of 2022.
- [93] In a similar vein, in his updated report, dated August 3, 2023, Dr. Ranalli reviewed the CNRs of Dr. Levitan and concluded that the applicant’s headaches appeared to have arisen from her cervical muscular strain/sprain injury. Yet, Dr. Ranalli provided no explanation on how he arrived at this conclusion when none of the other doctors have, and it is unclear why his previous report did not attribute the headaches to a cervical muscular strain/sprain injury, but his updated opinion did.
- [94] In any event, the applicant has reported to Drs. Karmy, Walton and Mr. Miller that she continues to have headaches, light sensitivity, and visual aura which affect

her self-employment because she is not working as many hours as she could if the impairment had not occurred. Dr. Karmy has also opined that as a result of the applicant's headaches and chronic pain, she has functional limitations with her housekeeping, recreational activities, and employment as an esthetician. Therefore, I find that it is reasonable and necessary to conduct both an otorhinolaryngology report and neurological assessment to assess whether she meets the threshold for a CAT impairment.

- [95] The applicant has also established that she sustained psychological impairments from the accident because I prefer the evidence of Dr. Miller and Mr. Walton over Dr. Azizli's report (dated December 2, 2021) because I find it is consistent with the CNRs of the applicant's family physician, Dr. Grieg.
- [96] The applicant has been diagnosed with an adjustment disorder with anxiety by Dr. Miller and Mr. Walton. They further opined that a combination of the applicant's psychological and difficulties with working under LED lights affected her ability to work in her self employment as an esthetician. Meanwhile, Dr. Azizli (in the report, dated December 2, 2021) concluded that the applicant sustained no significant psychological diagnosis and had returned to all activities of daily living and gainful employment, despite the applicant advising that she had not returned to snowboarding and had pain when working.
- [97] In any event, psychological complaints are documented in the CNRs of Dr. Grieg. For instance, on December 15, 2020, Dr. Grieg opined that the applicant had anxiety post-MVA. On December 21, 2020, Dr. Grieg noted that the applicant has been driving and that the anxiety had mostly gone away. However, on March 19, 2021, he noted that the applicant continued to have anxiety while driving on the highway. On June 23, 2021, Dr. Grieg noted that the applicant continued to have anxiety.
- [98] I find that the applicant has established she sustained a psychological impairment as a result of the accident because Dr. Miller and Mr. Walton's diagnosis of an adjustment disorder with anxiety is consistent with the applicant's complaints of psychological symptoms and Dr. Grieg's diagnosis of anxiety post-MVA. Dr. Azizli (in the report, dated December 2, 2021) also noted that the applicant scored in the marked range for negative affect and social withdrawal. Thus, I find that the proposed psychiatric assessment is reasonable and necessary to investigate whether the applicant's psychological impairments result in a CAT impairment in either criterion seven or eight.
- [99] I also find that the OT assessments are reasonable and necessary to investigate her functional abilities for the purpose of assessing criterion eight. Also, as noted

above, the applicant has consistently reported limitations with her employment, housekeeping tasks, and recreational activities, such as snowboarding, hockey and skating. Therefore, I find that the evidence indicates that the applicant suffered from functional impairments in several domains of her life, and that there is a sufficient basis upon which further investigation by way of OT assessments are warranted.

- [100] I also find that the orthopaedic assessment is reasonable and necessary because the applicant has reported neck and left shoulder pain to a vast majority of the assessors, and Dr. Grieg. I also place significant weight on Dr. Karmy's opinion that the applicant has sustained a chronic pain syndrome because the applicant has consistently reported that her pain has resulted in being unable to complete heavier housekeeping tasks, ice hockey, skating, and snowboarding. Dr. Grieg's CNRs also note restricted range of motion in the applicant's left shoulder.
- [101] Meanwhile, I place little weight on Dr. Boynton's report, dated December 2, 2021, where she opined that the applicant's minor residual symptoms would resolve over time because the applicant has testified in her affidavit, cross examination and reported to a vast majority of the assessors after this date that she continues to have neck and left shoulder pain. Likewise, I place little weight on Dr. Boynton's conclusion in her updated report, dated December 7, 2023, because she concluded that the applicant's myofascial pains were unrelated to the accident and due to poor posture, but she provided no justification on where this information came from. For these reasons, I find that the orthopaedic assessment is reasonable and necessary to determine whether the applicant's physical impairments would result in a CAT impairment.
- [102] As the applicant has established that it is reasonable and necessary to investigate whether her accident-related impairments result in a CAT impairment under criterion seven. I also find that the proposed AMA Rating Report is reasonable and necessary as criterion seven requires a rating methodology in accordance with the AMA Guides, 6<sup>th</sup> edition and 4<sup>th</sup> edition.
- [103] I also find the applicant is entitled to payment for the cost of completing the OCF-18 and OCF-19 pursuant to sections 25(1)(3)(iii) and 25(1)(5) of the *Schedule*.
- [104] For all these reasons, I find that the applicant is entitled to \$16,272.00 for completion of the documents and for the CAT assessments.

***The applicant is entitled to interest***

- [105] Pursuant to section 51 of the *Schedule*, the applicant is entitled to interest for the OT assessment and the CAT assessments.

***The respondent is liable to pay an award***

- [106] I find that the applicant is entitled to an award in the amount of 25%, and interest for the OCF-18s for an OT assessment and for the catastrophic impairment assessments.
- [107] Under s. 10 of O. Reg. 664, the applicant may be entitled to an award of an amount up to 50% of the benefits and interest owed to her if I find that the respondent unreasonably withheld or delayed payments.
- [108] The threshold for an award is high. However, I accept the applicant's submission that the respondent failed to consider the records of Dr. Leung and its own s. 44 assessor, Dr. Ranalli, which supported that the applicant sustained hearing loss in her left ear which is not a minor injury. Indeed, in an adjuster's log note dated April 29, 2021, the adjuster noted that the applicant's injury level was MIG, but under a subheading called "Escaped" noted "No- need medicals regarding hearing loss for consideration."
- [109] In my interpretation, the adjuster was acknowledging that the applicant could be removed from the MIG based on medical documentation being received to support she sustained hearing loss. The respondent did not address this argument raised by the applicant and, instead, argued that its denials were based on the medical evidence and the s. 44 reports. However, as noted above, Dr. Leung's CNRs supported that the applicant sustained hearing loss in her left ear, which as acknowledged by the respondent in its log notes, could allow the applicant to escape from the MIG. Based on the foregoing, I find that the respondent ignored the medical evidence before it and has not provided an explanation on why. The respondent has an obligation to review the medical documents before it and adjust its claim fairly. In ignoring the medical evidence, the respondent did not fulfil its obligation and unreasonably withheld benefits to the applicant.
- [110] I also find that the respondent acted unreasonably when it kept the applicant in the MIG based on the conclusion of s. 44 assessor, Dr. Ranalli because it simply followed the assessor's opinion without question despite hearing loss and a traumatic brain injury not being a minor injury under the *Schedule*. The respondent has provided no explanation on why it accepted Dr. Ranalli's

conclusion that a traumatic transient sensorineural hearing impairment is a minor injury, when it clearly does not fall within the definition of a minor injury. Likewise, the respondent accepted Dr. Ranalli's opinion that a mild closed head injury (grade I or Grade II) is in the MIG, despite a clear reading of s. 3(1) would indicate otherwise.

- [111] In determining the quantum of a special award, it is well-settled that the Tribunal has found that the following factors may be considered: (i) the blameworthiness of the respondent's conduct; (ii) the vulnerability of the applicant; (iii) the harm or potential harm directed at the applicant; (iv) the need for deterrence; (v) the advantage wrongfully gained by the insurer from the misconduct; (vi) should take into account any other penalties or sanctions that have been or likely will be imposed on the insurer due to its misconduct; and (vii) the overall length of the delay.
- [112] In this case, not considering the applicant's hearing loss or traumatic brain injury when determining her status under the MIG provided a monetary advantage to the respondent. The conduct of ignoring medical evidence and relying on s. 44 assessors' opinion without question or consideration should be deterred. Without further information, I cannot comment on the applicant's vulnerability or harm directed at her. I do not believe that the respondent's behaviour attracts the full 50% maximum award allowable under O. Reg. 664. As noted above, I have determined that the applicant is entitled to the OT assessment and entitled to the OCF-18 for CAT impairment assessments. I find that 25% is an appropriate quantum for the award, plus interest pursuant to O. Reg. 664.

## ORDER

- [113] For the reasons outlined above, I find that:
- a. The applicant is removed from the MIG.
  - b. The applicant is entitled to the OCF-18s for an OT assessment and for the catastrophic impairment assessment plus interest;
  - c. The applicant is not entitled to the OCF-18 for medical services (MRI and SPECT Scan); and

- d. The applicant is entitled to an award in the amount of 25 percent, plus interest pursuant to s. 10 of Reg. 664 for the OCF-18s for an OT assessment and for the catastrophic impairment assessments.

**Released:** February 6, 2025

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**Tanjoyt Deol**  
Adjudicator